

Hives - Female (Child)

Algorithm 1396 | Child (0-17 years) | Gender at birth: Female

Definition

- UAT-only assessment of suspected hives (urticaria), with an emergency safety screen for anaphylaxis; not approved for real-patient or production use.
- Child patient.
- Female.

Initial assessment

1. [OPEN_TEXT] What is the patient's exact age, when did the rash begin, and what does it look and feel like?
2. [YES_NO] Is there swelling of the lips, mouth, tongue or throat; difficult or noisy breathing; tight throat; difficulty swallowing; wheeze; dizziness; collapse; confusion; unusual drowsiness or floppiness?
3. [OPEN_TEXT] Was there a possible trigger such as food, medicine, insect sting, heat, cold, pressure, or infection, and does the patient have a prescribed allergy action plan or adrenaline auto-injector?
4. [YES_NO] Is there high fever or marked illness, painful or blistering skin, purple spots that do not fade with pressure, eye or mouth involvement, or swelling beneath the skin?
5. [CAREGIVER_OBSERVATION] What has a safe parent or caregiver observed about alertness, breathing, pain, swelling, fever, eating or drinking, sleep, play, and normal activity?

Triage and redirect questions

1. Redirect → Anaphylaxis

Are there breathing problems, throat tightness, collapse, or swelling of the lips, mouth, tongue, or throat?

Hives with airway, breathing, or circulation features belong in the Anaphylaxis pathway.

1. Disposition level 100

Is there sudden swelling of the lips, mouth, tongue or throat; tight throat, hoarse voice or difficulty swallowing; fast, noisy or difficult breathing or wheeze; blue, grey or very pale colour; cold or clammy skin; severe dizziness, confusion, collapse or reduced responsiveness; or unusual floppiness in an infant or child? Treat these features as suspected anaphylaxis even if the hives are mild or absent.

The corrected Anaphylaxis pathway and 2025 Resuscitation Council UK guidance recognize airway, breathing or circulation compromise as anaphylaxis with or without skin changes. HMC directs severe allergic reactions in Qatar to 999.

1. Disposition level 70

Only after anaphylaxis and other emergency rash features are excluded, have hives failed to improve after 2 days, spread or recurred, or occurred with fever, feeling unwell, caregiver concern about a child, or swelling beneath the skin?

NHS.UK guidance recommends urgent clinical review for these features; the local Qatar urgent-care pathway is used here.

1. Disposition level 15

Are these typical itchy, raised hives in a patient who is otherwise well, with all emergency and urgent features excluded and no concern about a young child?

NHS.UK guidance describes hives as typically resolving within days with self-treatment.

Advice

Suspected anaphylaxis – call Qatar 999

- Call Qatar 999 for an ambulance immediately and follow the Anaphylaxis emergency pathway. If the patient has their prescribed adrenaline auto-injector, help them use it promptly according to the device instructions without delaying 999. Keep the patient lying flat with legs raised if tolerated; if breathing is difficult, allow sitting with legs extended. Do not let the patient stand or walk. If symptoms persist after 5 minutes and a second prescribed device is available, use it. Antihistamines do not treat the airway, breathing, or circulation features of anaphylaxis and must not delay adrenaline or 999. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: symptoms persist after the first auto-injector; breathing worsens; collapse or reduced responsiveness.

Same-day hives assessment

- Arrange same-day clinical assessment through the Qatar urgent-care destination approved for this UAT environment. Keep the patient observed and escalate immediately to Qatar 999 if any airway, breathing, circulation, or reduced-responsiveness feature develops.
- Call Back If: swelling of lips, mouth, tongue, or throat; breathing or swallowing difficulty; dizziness, collapse, or unusual floppiness.

Isolated hives after emergency screening

- Avoid a suspected trigger when this can be done safely. A pharmacist or clinician should confirm whether an antihistamine is suitable and select the product, formulation, and dose using the patient's age, weight, pregnancy or breastfeeding status, other medicines, and health conditions. Do not give an adult product or dose to a child. Continue observation and use Qatar 999 immediately if anaphylaxis features develop.
- Call Back If: swelling of the lips, mouth, tongue, or throat; breathing or swallowing difficulty; hives do not improve after 2 days or recur.

First aid

- Call Qatar 999 for an ambulance immediately and follow the Anaphylaxis emergency pathway. If the patient has their prescribed adrenaline auto-injector, help them use it promptly according to the device instructions without delaying 999. Keep the patient lying flat with legs raised if tolerated; if breathing is difficult, allow sitting with legs extended. Do not let the patient stand or walk. If symptoms persist after 5 minutes and a second prescribed device is available, use it. Antihistamines do not treat the airway, breathing, or circulation features of anaphylaxis and must not delay adrenaline or 999. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- Hamad Medical Corporation, "Life-Threatening Medical Emergency", <https://hamad.qa/EN/Emergency/Pages/Life-ThreateningMedicalEmergency.aspx> (accessed 25 July 2026)
- Resuscitation Council UK, "First Aid Guidelines â€” Anaphylaxis", 2025, <https://www.resus.org.uk/professional-library/2025-resuscitation-guidelines/first-aid-guidelines>
- NHS.UK, "Hives", <https://www.nhs.uk/conditions/hives/>
- NHS.UK, "Antihistamines", <https://www.nhs.uk/medicines/antihistamines/>

Provenance

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